SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Vinorelbine and cisplatin · 21-day cycle

Postoperative adjuvant or advanced/recurrent NSCLC: confirm your treatment goal · NCCP 00339 · version 7 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
After surgery, treatment aims to lower the chance of cancer returning. For advanced or recurrent lung cancer, it aims to control disease and symptoms. Your team should mark which goal applies to you; benefit is not guaranteed.

Schedule
A cycle lasts 21 days (3 weeks). Vinorelbine is given on Days 1, 8, 15. Cisplatin is given on Day 1. Four cycles are planned, with review before doses. Delays or omitted doses may be needed for safety.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Vinorelbine
VINCA ALKALOID CHEMOTHERAPY

- ☐ Low blood counts and serious infection
- ☐ Constipation; rarely bowel obstruction
- ☐ Numbness or tingling
- ☐ Infusion-site injury if medicine leaks
- ☐ Fatigue, nausea and hair thinning

Cisplatin
PLATINUM CHEMOTHERAPY

- ☐ Low blood counts, infection and bleeding
- ☐ Nausea, vomiting and fatigue
- ☐ Mouth soreness and diarrhoea
- ☐ Kidney problems
- ☐ Allergic or infusion reactions
- ☐ Hearing changes or tinnitus, which can persist
- ☐ Numbness or tingling, which can persist
- ☐ Electrolyte changes and need for hydration
- ☐ Effects on fertility

Follow your prescribed anti-sickness and constipation-prevention plan. Cisplatin days need kidney-protecting fluids and electrolyte checks. Do not repeat the full cisplatin anti-sickness course on a vinorelbine-only day unless prescribed. Discuss serious infection, hearing or nerve damage that can persist, kidney injury, fertility and rare life-threatening effects.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00339 v7.

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